PBM FAQ for Executives

Twenty questions, and straight answers, on pharmacy benefit oversight

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AUDIENCE	Executives, board members, and benefit leaders responsible for a plan they did not negotiate
PURPOSE	A crash course in what a PBM contract does, what it hides, and what a fiduciary is obliged to ask
COMPANION	Contract X-Ray scores an individual contract. The CAA 2026 Readiness Report publishes how the market scores overall
ONLINE	nautilushealth.org/pbm · contractxray.com

What Executives Should Know About PBMs

Pharmacy Benefit Managers (PBMs) control more than just your drug spend. They control access, pricing, and outcomes.

Yet many executives don't know who their PBM is, how they're paid, or whether they're harming the very people the plan is meant to serve. This FAQ is your crash course in PBM oversight, designed to arm executives, board members, and benefit leaders with the essential questions (and straight answers) they need to cut through the confusion, uncover hidden risks, and take back control.

1. What is a PBM and why should employers care?

PBMs administer prescription drug plans. They control access, pricing, and pharmacy reimbursements, yet they often operate with minimal transparency, impacting plan costs and participant outcomes.

2. How concentrated is the PBM market?

Three PBMs, CVS Caremark, Express Scripts, and OptumRx, control over 80% of the market. Their dominance limits competition and transparency that often drive up employer costs.

3. Why are employers being sued over PBM contracts?

Lawsuits allege employers failed to monitor PBM practices and allowed excessive costs. Several early cases were dismissed on standing rather than on the merits, so the question of what adequate oversight looks like remains open. The underlying duty does not: employers are plan fiduciaries under ERISA, and that obligation extends fully to the pharmacy benefit.

4. Are advisors and consultants trustworthy guides?

Not always. Many receive hidden commissions or bonuses from PBMs, creating conflicts of interest. Federal law now requires disclosure, but employers must demand and verify it.

5. What is 'spread pricing' and why is it a problem?

Spread pricing occurs when PBMs charge the plan more than they reimburse pharmacies. The PBM keeps the difference, creating incentives to maximize spread rather than control costs.

6. What's the deal with rebates?

Rebates from drugmakers often go to PBMs instead of the plan. They inflate list prices and incentivize high-cost drugs over lower-cost alternatives. Employers should demand 100% rebate pass-through.

7. What is 'carving out' the PBM, and why does it matter?

Most PBMs are bundled with your medical carrier or TPA. Carving out means selecting an independent PBM using fiduciary criteria, which brings transparency, competition, and oversight.

8. What is the First Principle: Right Drug > Right Participant > Right Price?

It means plans must prioritize clinical appropriateness first, then equitable access, then cost. Cutting corners or chasing rebates can harm participants and violate fiduciary duties.

9. What does a fiduciary-aligned PBM contract contain?

Ten provisions across three domains. **Fiduciary Conduct:** fiduciary loyalty, conflict of interest and neutrality, lowest net cost and clinical integrity. **Financial Integrity:** pass-through pricing, rebate and manufacturer revenue, administrative fee transparency. **Oversight and Control:** data ownership, audit rights, carve-out and vendor rights, termination and clean exit. Each is scored from 0 to 100 against published model contract language, and the ten together produce the Fiduciary Alignment Score.

10. Can PBMs be fiduciaries?

Yes. If they exercise discretion over plan assets, such as setting pricing or denying claims, they may be fiduciaries under ERISA, exposing them (and you) to liability.

11. Are employers required to monitor their PBM?

Absolutely. ERISA requires prudent oversight of all vendors, including PBMs. Ignorance is not a defense, and documentation of your oversight is essential.

12. What should employers demand in a PBM contract?

Clear definitions, full data access, audit rights, 100% rebate pass-through, no spread pricing, and performance guarantees. Ambiguous terms cost money, and they invite lawsuits.

13. What is the 'lowest net cost' approach?

Instead of chasing rebates, fiduciaries must evaluate drugs based on total cost to the plan, after discounts, rebates, and fees. High rebates don't always mean lower net costs.

14. How can employers run a fiduciary-aligned RFP?

Use open-source RFP templates with clearly defined requirements. Include scorecards, contract terms, and evaluation rubrics that align with fiduciary best practices.

15. How much do PBMs make, and how do they get paid?

Many PBMs profit from hidden markups, rebate retention, and spread pricing. True fiduciary-aligned PBMs earn a flat fee. If you can't follow the money, you're likely overpaying.

16. What's the role of data in PBM oversight?

If you don't have full claims and pricing data, you can't verify compliance or performance. Fiduciary oversight requires independent analysis, not trust.

17. What is Contract X-Ray?

It is an independent service that scores a PBM contract against the ten provisions above and returns a Fiduciary Alignment Score from 0 to 100, provision by provision, with a citation behind every finding. A free Quick Look rates all ten. A Full Assessment adds a Contract Scorecard, a Negotiation Report with the language to propose, an Executive Brief, and a Data Sovereignty Score.

18. What's the downside of doing nothing?

Rising costs, poor outcomes, fiduciary risk, and reputational exposure. Early cases turned on standing rather than on whether oversight was adequate, so the litigation picture is unsettled. The duty is not. A plan sponsor who cannot say what its contract permits cannot show it exercised prudent oversight.

19. How can employers start fixing this problem?

1) Educate fiduciaries. 2) Demand contract transparency. 3) Launch a Contract Compliance RFI. 4) Review rebate flows and pricing. 5) Document your decisions.

20. What if our advisor resists PBM changes?

Ask who pays them. Demand full compensation disclosure. If it's the PBM, not you, they are conflicted. Fiduciaries must prioritize plan participants, not advisor convenience.

KEY TAKEAWAY

The PBM model is not broken. It is working exactly as designed. The problem is, it wasn't designed for you.

Executives have the power, and the duty, to change that. By demanding transparency, rejecting conflicts, and insisting on fiduciary-aligned contracting, you can transform your pharmacy benefits from a black box to a strategic asset.

It starts with asking better questions. And not backing down until you get real answers.

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